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COURSE SLIDES · NON-WSQ COURSE

Improve Employee Wellness and Work Resilience



Course Code: C1158

Conducted by Tertiary Infotech Academy Pte Ltd

Version v1.0 · 2 October 2026

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COURSE ADMINISTRATION

Welcome and Course Overview

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COURSE PROVIDER

About the Trainer



Tertiary Infotech Academy

Training provider

ORGANISATION

Tertiary Infotech Academy Pte Ltd

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201200696W

COURSE

Improve Employee Wellness and Work Resilience

COURSE CODE

C1158

PROGRAMME

Professional short course

CONTACT

enquiry@tertiaryinfotech.com

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YOUR TRAINER

About the Trainer



Dr. Alfred Ang

Principal Trainer · Tertiary Infotech Academy Pte Ltd

ROLE
Principal Trainer and course facilitator

SPECIALISMS
Workplace wellbeing, responsible technology and programme management

DELIVERY
Professional development and workplace application

CONTACT
enquiry@tertiaryinfotech.com

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INTRODUCTION

Let's Know Each Other

1

Role

Your organisation, function and wellbeing responsibilities.

2

Employee experience

One workplace condition that helps or drains sustainable energy.

3

Learning goal

One programme decision or capability you want to strengthen.

4

Case lens

One employee group whose access needs you will represent.

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HOUSEKEEPING

Ground Rules

1. **Participate**
   Challenge assumptions and contribute evidence.

2. **Respect**
   Disagree with ideas, not people.

3. **Confidentiality**
   Use public or course data only; do not disclose live incident information.

4. **Devices**
   Silent mode; step out quietly if needed.

5. **Time**
   Return from breaks promptly.

6. **Attendance**
   Attend the full day to get the most from the course.

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LESSON PLAN

Course Outline — 1 Day

Morning

- Morning — Understand the evidence

– Topic 1: requirements, factors and wellbeing dimensions

– Topic 1: trends, technology and evidence

– Activity 1: privacy-safe wearable pilot

– Activity 2: segmented workforce needs

Afternoon

- Afternoon — Design, implement and improve

– Activity 3, then Topic 2: programme design and implementation

– Activities 4–7: resilience, portfolio, playbook, participation

– Topic 3: communication, feedback and evaluation

– Activities 8–10 and course wrap-up

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HANDS-ON MATERIALS

Access the Course Activities

Open resource · github.com

Canonical source

The repository holds the current learner-facing activity package.

Individual folders

Each activity keeps its scenario, data, questions and evidence together.

Detailed guidance

Procedures and acceptance criteria are in the Learner Guide and activity Markdown.

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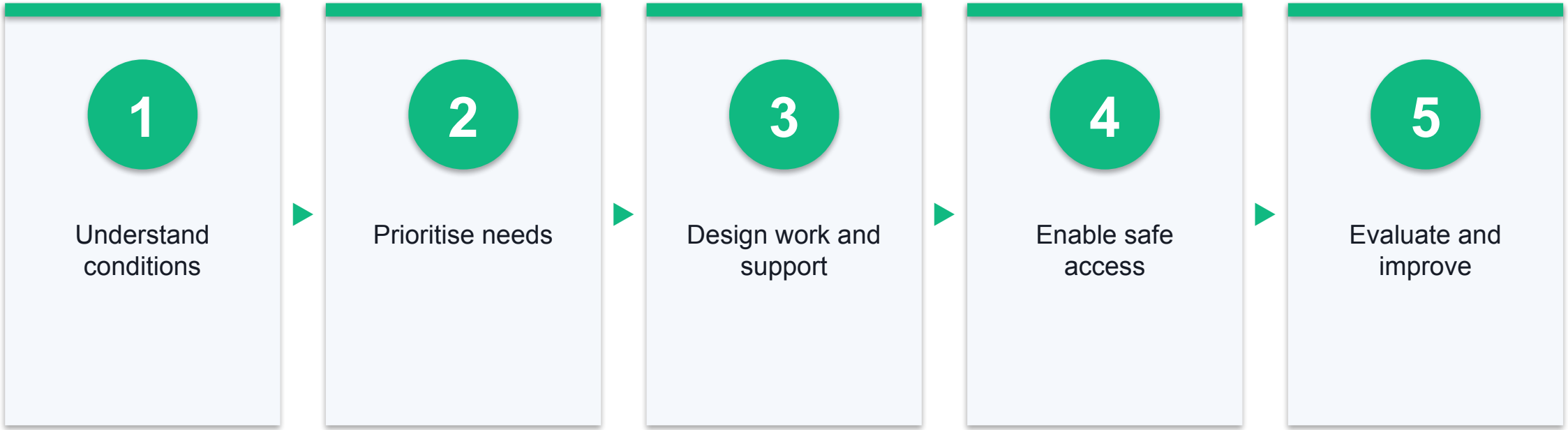
CORE MODEL

Understand · Design · Enable · Learn

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END-TO-END VIEW

The workplace wellbeing system



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EVIDENCE BEFORE CONFIDENCE

Evidence hierarchy

1

Verified public fact

Published or regulatory information with source and date.

2

Organisation evidence

Workforce voice, risk data, programme records and approved decisions.

3

Training assumption

A simulated input used only for learning and clearly labelled.

4

Pilot observation

Observed behaviour or artefact compared with pre-set criteria.

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COURSE MODEL

Employee wellbeing emerges from the whole system.

Leadership, job design, access, trust and personal resources must work together.

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01

LEARNING UNIT 01

Workplace Health, Wellness and Evidence

wellbeing dimensions · Singapore requirements · needs · trends · technology · evidence

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KEY CONCEPTS

Key Concepts — Workplace Health, Wellness and Evidence

1

Health

A state and capability shaped by physical, mental and social conditions—not merely the absence of illness.

2

Wellness

Intentional practices and enabling conditions that support people to function and thrive.

3

Evidence

A useful decision separates verified facts, workforce data, professional judgement and assumptions.

4

System

Job design, managers, culture, access and individual resources interact; a perk alone is not a strategy.

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TOPIC 01

Wellbeing is an operating condition, not a collection of perks.

Design work and support so that people can be healthy, contribute and recover sustainably.

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TOPIC 01

Health and wellness: related, not interchangeable

Perspective A

- Health includes physical, mental and social functioning and limitations.
- It may require clinical care, reasonable support and risk controls.
- Employers should not diagnose or infer a condition from behaviour.

Perspective B

- Wellness describes choices, resources and conditions that support functioning.
- Participation should be voluntary, inclusive and free from coercion.
- Programme design complements—not replaces—safe work and medical care.

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TOPIC 01

Interdependent dimensions of wellbeing

1

Career

Meaning, manageable expectations, strengths and development.

2

Social

Belonging, support, respect and constructive relationships.

3

Financial

Security, capability and access to trustworthy guidance.

4

Physical

Energy, movement, rest, nutrition and safe work.

5

Community

Connection, contribution and confidence in the wider environment.

6

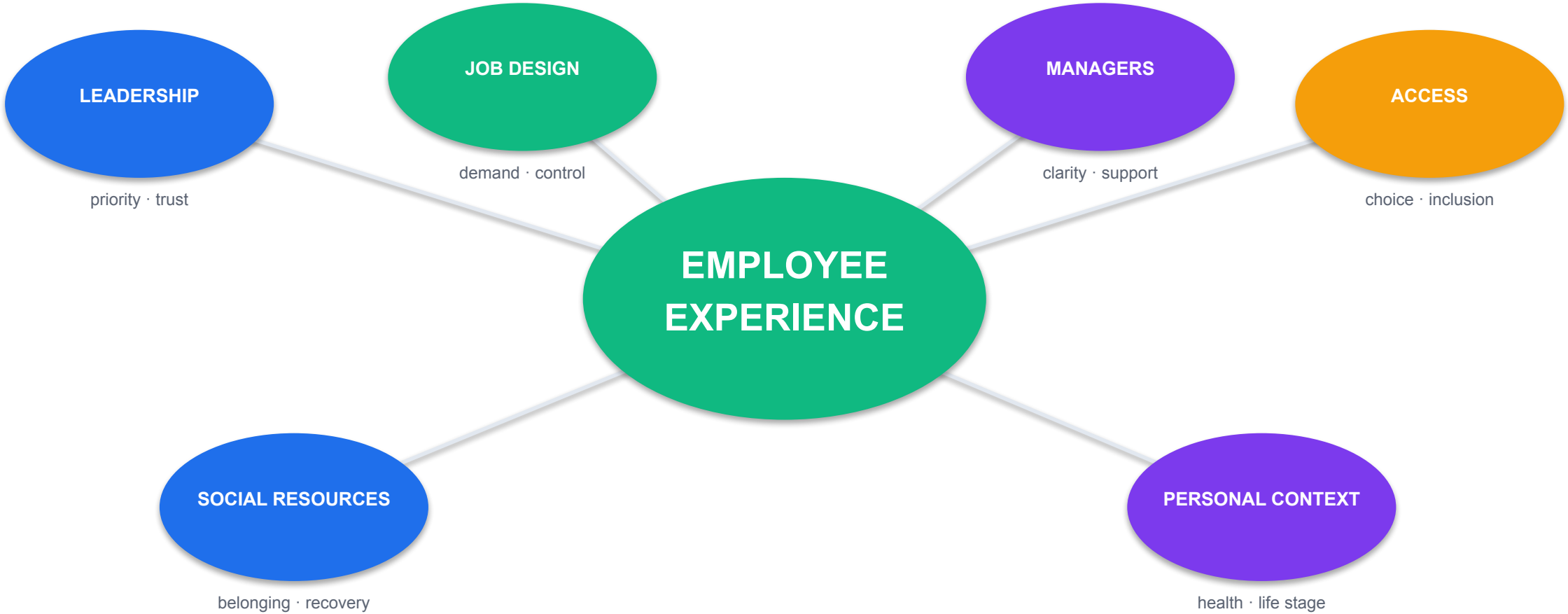
Mental and emotional

Psychological safety, recovery, coping resources and access to help.

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SYSTEM VIEW

The workplace wellbeing system



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TOPIC 01

Population health indicators and workplace decisions

Perspective A

- Life expectancy estimates years lived; health-adjusted life expectancy also considers years lived in less than full health.
- Population indicators can signal prevention priorities and health inequality.
- They do not diagnose a workforce or measure one employer programme.

Perspective B

- Use workforce needs, job risks, access and employee voice for workplace decisions.
- Do not convert population averages into individual targets or employment judgements.
- Connect prevention with safe work, timely support and public-health pathways.

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TOPIC 01

Singapore workplace requirements: the practical baseline

1

PDPA

Use a lawful purpose, notify appropriately, protect data, limit retention and enable applicable access/correction rights.

2

Workplace safety and health

Manage workplace risks, including work conditions that can harm physical or psychological health.

3

Employment practices

Make objective decisions, maintain confidentiality and provide fair, respectful support.

4

Future fairness regime

Prepare for Singapore's Workplace Fairness Act implementation; do not describe future duties as already in force.

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TOPIC 01

Singapore health and employment touchpoints

1

Employment Act sick leave

Eligible employees have statutory outpatient and hospitalisation leave subject to service, notification and certification rules.

2

Healthier SG

A national preventive-health initiative centred on family doctors, health plans and community support; employers can signpost without accessing clinical data.

3

HPB workplace support

Current workplace programmes and tools can complement an organisation's needs-led plan; validate eligibility and terms before commitment.

4

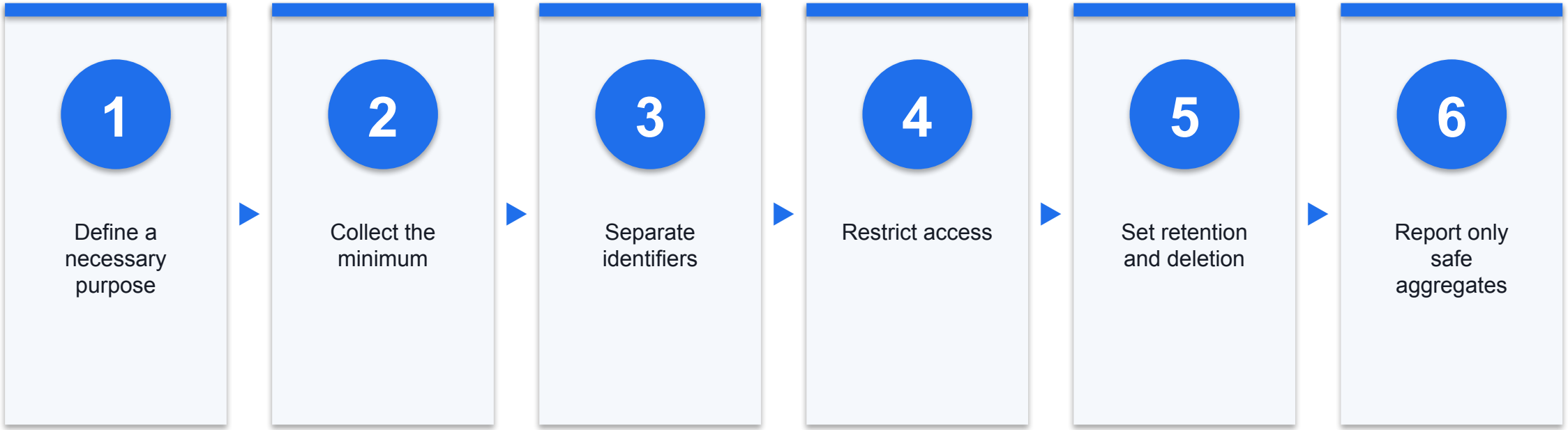
Employment data boundary

Administer leave and benefits with necessary information; do not infer a blanket right to employee diagnoses or unrelated health data.

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TOPIC 01

Privacy-by-design for wellbeing data



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TOPIC 01

Consent and power imbalance

Perspective A

- A checkbox does not make every use fair or necessary.
- Employees may feel pressure when managers sponsor participation.
- Avoid linking sensitive health disclosure to performance decisions.

Perspective B

- Offer a genuine choice and a useful non-participation path.
- Explain purpose, recipients, retention and withdrawal clearly.
- Use independent providers and aggregates where practical.

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TOPIC 01

Data risks in wearables and wellness platforms

1

Function creep

Step or sleep data becomes input for unrelated employment decisions.

2

Re-identification

Small teams or rare conditions reveal identities despite aggregation.

3

Vendor exposure

Cloud, subcontractor and cross-border processing expands the risk surface.

4

Exclusion

Device, language, disability or algorithm design disadvantages some groups.

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TOPIC 01

Factors that shape employee health

1

Work design

Demand, control, staffing, schedules, role clarity and workload variability.

2

Relationships

Manager behaviour, team norms, bullying, support and psychological safety.

3

Personal context

Health, caring duties, finances, life stage and access to services.

4

Environment

Ergonomics, noise, heat, travel, food options and digital load.

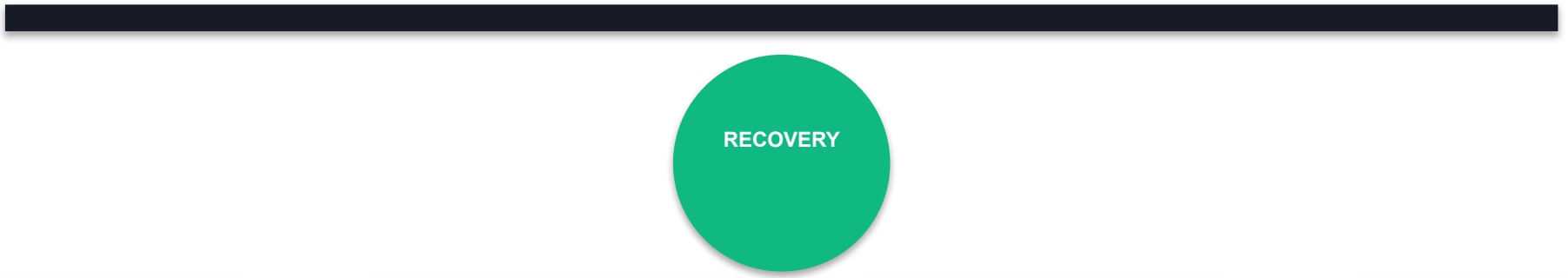
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WORK DESIGN

Job Demands–Resources: redesign the balance

DEMANDS
workload · emotion · uncertainty · time

RESOURCES
control · clarity · support · recovery



REMOVE
preventable hindrance

RESOURCE
necessary challenge

RECOVER
protected time

REVIEW
employee + service evidence

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TOPIC 01

Mental-health continuum: respond without diagnosing

1

Thriving

Sustainable energy, connection and effective functioning.

2

Struggling

Observable changes in work, mood, energy or connection may need a supportive check-in.

3

Unwell

Persistent impairment may need professional support and workplace adjustment.

4

Crisis

Immediate safety concerns require the organisation's emergency and escalation protocol.

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TOPIC 01

Supportive observation versus diagnosis

Perspective A

- Describe observable changes: missed deadlines, withdrawal or unusual fatigue.
- Ask privately, listen, clarify work needs and signpost support.
- Escalate immediate risk using the approved protocol.

Perspective B

- Do not label anxiety, depression, bipolar disorder, OCD, psychosis, eating disorders or substance misuse.
- Do not promise absolute secrecy when safety action may be necessary.
- Do not collect clinical details that the work decision does not require.

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TOPIC 01

Needs-assessment evidence

1

Workforce voice

Anonymous survey, focus groups, interviews and employee-resource groups.

2

Operational data

Absence, overtime, turnover, incidents, workload and schedule patterns.

3

Service data

Aggregated EAP or benefit utilisation and access barriers.

4

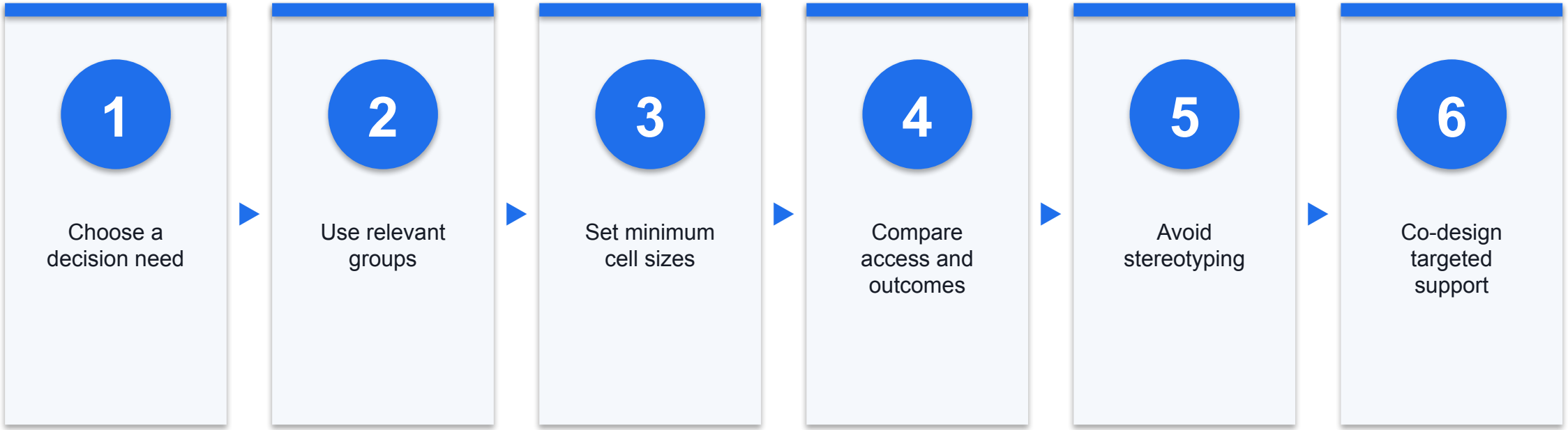
Risk evidence

WSH assessments, ergonomic observations and job-demand reviews.

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TOPIC 01

Responsible segmentation



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TOPIC 01

Workplace wellbeing trends

1

Whole-person design

Connect physical, mental, social, financial and career support.

2

Hybrid access

Blend onsite, remote, synchronous and self-directed pathways.

3

Personalisation

Offer choice while protecting privacy and accessibility.

4

System accountability

Measure manager and job-design conditions, not only individual habits.

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TOPIC 01

Technology options and trade-offs

1

Wearables

Continuous feedback can motivate, but creates surveillance and accuracy risks.

2

Telehealth

Improves reach, but depends on clinical quality, privacy and digital access.

3

Apps and gamification

Can support habits; novelty, competition and accessibility may limit benefit.

4

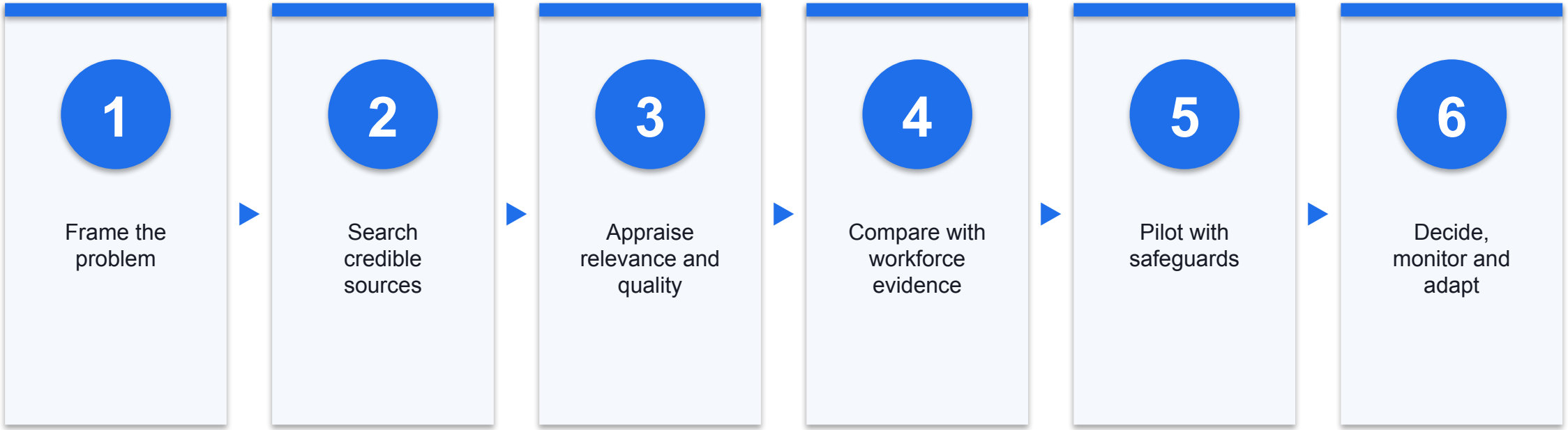
AI coaching

Scales prompts and navigation; requires safety boundaries, bias checks and human escalation.

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TOPIC 01

Evidence-to-decision pipeline



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TOPIC 01

A fashionable tool is not a wellbeing outcome.

Choose technology only when a defined need, safeguard and measure justify it.

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REAL-USE-CASE DECISIONS

Applied Activities — Workplace Health, Wellness and Evidence

1

Activity 1

Privacy-Safe Wearable Wellness Pilot · LO1

2

Activity 2

Segmented Workforce Wellbeing Needs Assessment · LO2

3

Activity 3

Evidence and Technology Selection Board · LO1

4

Evidence boundary

Published evidence and requirements are separated from fictional workplace inputs.

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LEARNING UNIT 01 · LO1

Privacy-Safe Wearable Wellness Pilot

ACTIVITY 1

Assess whether a voluntary wearable pilot can address a defined employee need without creating surveillance, exclusion or unsafe data use.

You'll build

Data-flow map, privacy-and-equity risk register and conditional pilot decision

Tools / evidence: PDPC obligations · wearable vendor fact sheet · employee personas · pilot decision canvas

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ACTIVITY 1 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- Singapore's PDPA sets obligations including accountability, notification, consent, purpose limitation, accuracy, protection, retention limitation, transfer limitation and applicable access/correction rights. These obligations shape the design of employer-sponsored wellbeing data flows.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- MarinaLink Logistics, all workforce figures and vendor terms are fictional training inputs.
- The proposed device records steps, heart rate, sleep estimates, location during challenges and a user identifier.
- Participation is advertised as voluntary, but line managers will distribute devices and see individual dashboards under the draft plan.

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ACTIVITY 1 · DISCUSS AND DECIDE

Activity Questions

1. What decision purpose is necessary and proportionate, and which proposed fields are excessive?
2. Where could power imbalance undermine voluntariness or trust?
3. Which privacy, accessibility and fairness controls are pilot gates?
4. Should the pilot proceed, proceed conditionally or stop—and what evidence supports the decision?

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ACTIVITY 1 · ACCEPTANCE TEST

Privacy-Safe Wearable Wellness Pilot

✅ Test it

The submission defines a necessary purpose, minimises fields, removes manager access to individual data, offers a genuine alternative, specifies reporting thresholds and assigns every residual risk to an owner and gate.

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Segmented Workforce Wellbeing Needs Assessment

ACTIVITY 2

Combine workforce voice and operational signals to prioritise needs without diagnosing employees or exposing small groups.

You'll build

Segmented needs matrix, evidence-quality log and priority statement

Tools / evidence: Gallup wellbeing model · fictional pulse data · focus-group extracts · operational indicators

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ACTIVITY 2 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- Gallup frames wellbeing through career, social, financial, physical and community elements and links the employee experience to outcomes such as engagement, absence and retention. The framework is a lens for inquiry, not a diagnostic instrument.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- All survey, absence, overtime, turnover and focus-group data in the activity pack are fictional.
- Groups with fewer than ten responses must not be reported separately.
- Operational indicators have multiple possible causes and cannot diagnose health conditions.

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ACTIVITY 2 · DISCUSS AND DECIDE

Activity Questions

1. What can each evidence source support, and what can it not support?
2. Which needs are shared and which differ by workforce group?
3. Where could response bias, small numbers or stereotyping distort the result?
4. Which two priorities should management address first, and why?

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ACTIVITY 2 · ACCEPTANCE TEST

Segmented Workforce Wellbeing Needs Assessment

✅ Test it

Every priority uses at least two evidence signals, small groups are suppressed, uncertainty is explicit, no condition is diagnosed, and recommended action addresses a workplace factor plus an access need.

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Lunch Break

30 minutes

Evidence and Technology Selection Board

ACTIVITY 3

Compare wearables, telehealth, wellbeing apps, gamification and AI coaching against evidence, need, safety, inclusion and implementation fit.

You'll build

Evidence appraisal table, weighted option matrix and pilot hypothesis

Tools / evidence: NCBI programme evidence · McKinsey research · technology briefs · appraisal rubric

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ACTIVITY 3 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- The NCBI review describes employee wellness programmes as employer-sponsored efforts to support healthy behaviour and reduce health risk, and stresses leadership, alignment, scope, accessibility, partnerships and communication. It also notes participation barriers such as awareness, time, manager support, perceived benefit, access and privacy.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- Vendor claims, prices and implementation times are fictional and have not been independently validated.
- The workforce need is access to timely support and recovery, not increased monitoring.
- Mobile access and English literacy vary across groups.

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ACTIVITY 3 · DISCUSS AND DECIDE

Activity Questions

1. Which source claims are credible, relevant and transferable to this workforce?
2. What employee need does each option address—and what new risk does it create?
3. Which criteria and weights should govern the choice?
4. What pilot result would justify scale, adaptation or termination?

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ACTIVITY 3 · ACCEPTANCE TEST

Evidence and Technology Selection Board

✅ Test it

The recommendation distinguishes research from vendor claims, maps each option to a defined need, includes privacy and accessibility gates, tests weight sensitivity and states scale/adapt/stop criteria.

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LEARNING UNIT RECAP

Recap — Workplace Health, Wellness and Evidence

1

Privacy-Safe Wearable Wellness Pilot

Apply legislative and ethical requirements to a workplace wellness technology decision

2

Segmented Workforce Wellbeing Needs Assessment

Identify the current health and wellness state of different employee groups using privacy-safe evidence

3

Evidence and Technology Selection Board

Identify and appraise wellbeing trends, technologies and research for workplace relevance

4

Evidence habit

Separate public facts, organisation evidence and training assumptions.

5

Management habit

Link every priority, risk or finding to an owner and review trigger.

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02

LEARNING UNIT 02

Programme Design and Implementation

programme portfolio · inclusion · policy-to-practice · resilience · participation · governance

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Key Concepts — Programme Design and Implementation

1

Portfolio

Combine prevention, protection, early help, support and recovery rather than relying on one programme.

2

Fit

Design for employee needs, operating context, access barriers and cultural expectations.

3

Implementation

Turn policy into roles, routines, resources, escalation and evidence.

4

Resilience

Build individual, team and organisational capacity while reducing preventable demands.

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TOPIC 02

Start with the work, then add support.

A resilience workshop cannot compensate for chronic overload, unclear priorities or unsafe management behaviour.

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TOPIC 02

Programme portfolio

1

Prevention

Healthy work design, risk reduction, ergonomics, financial capability and connection.

2

Promotion

Movement, nutrition, sleep, learning and inclusive social participation.

3

Early help

Manager conversations, screening boundaries, navigation and EAP access.

4

Treatment and recovery

Benefits, clinical pathways, adjustments and return-to-work support.

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TOPIC 02

Common programme types

1

Physical wellbeing

Activity, ergonomics, preventive checks, nutrition and chronic-condition support.

2

Mental wellbeing

Psychological safety, EAP, counselling, peer support and manager capability.

3

Financial wellbeing

Budgeting, debt support, benefits navigation and retirement planning.

4

Social and community

Connection, volunteering, inclusion and team practices.

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TOPIC 02

Disease management and prevention

Perspective A

- Disease-management support serves employees with an identified condition.
- It may include navigation, medication adherence and work adjustment.
- Clinical decisions belong with qualified professionals.

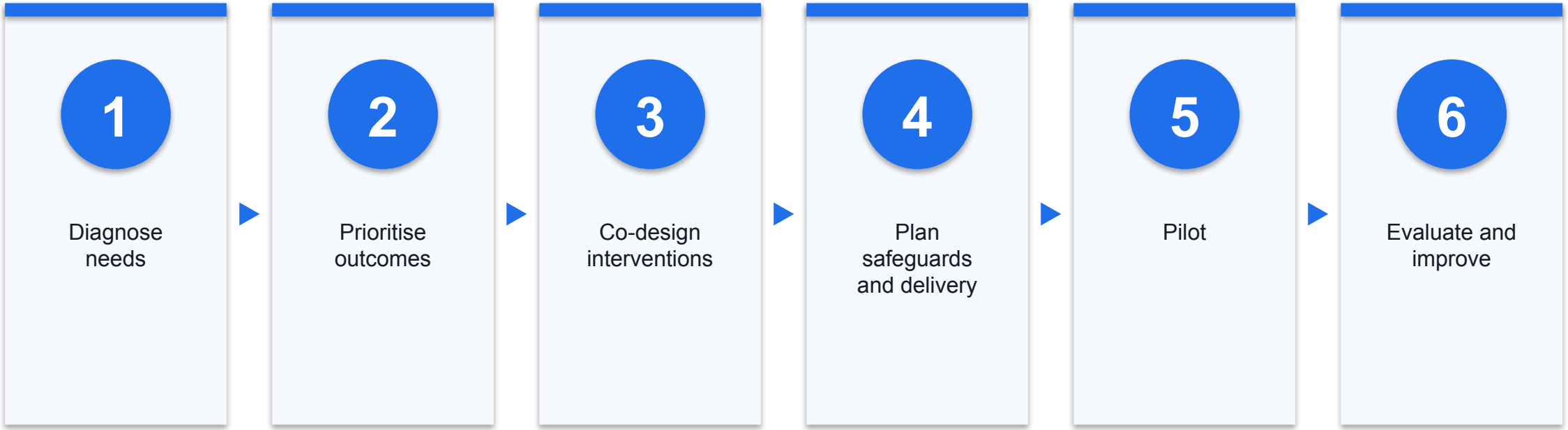
Perspective B

- Prevention reduces risk and strengthens protective conditions before harm.
- It includes job design, safe work, vaccination access and healthy environments.
- Universal support can be combined with targeted pathways.

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TOPIC 02

Programme design cycle



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TOPIC 02

Quality design criteria

1

Relevant

Addresses a material, evidenced need and defined employee group.

2

Accessible

Works across shift, location, language, disability, caregiving and device constraints.

3

Safe

Protects privacy, clinical boundaries, voluntariness and escalation.

4

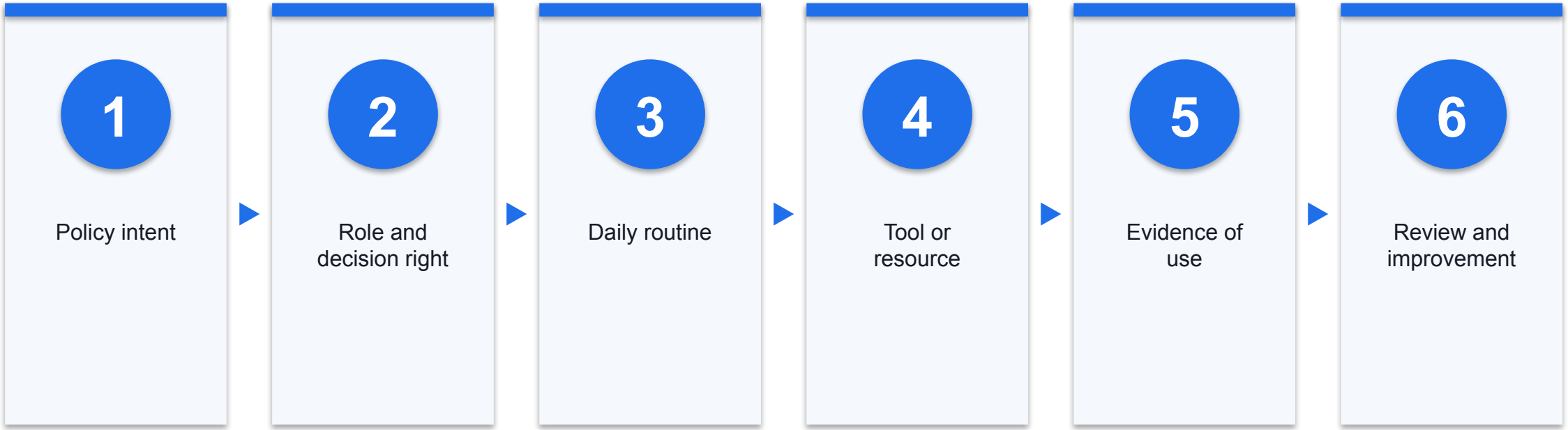
Evaluable

Has baseline, logic, owner, measures and review timing.

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TOPIC 02

Policy-to-practice chain



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TOPIC 02

Manager playbook essentials

1

Notice

Recognise work-related changes without diagnosing.

2

Ask

Choose a private moment and use open, respectful questions.

3

Act

Adjust work within authority and connect the employee to support.

4

Escalate

Follow safety, HR and emergency routes for risk or complexity.

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TOPIC 02

Individual resilience and organisational resilience

Perspective A

- Self-efficacy, optimism, recovery skills and social support can be protective.
- Practice, coaching and reflection can build usable coping resources.
- Participation should not imply blame for harmful work conditions.

Perspective B

- Leaders shape workload, autonomy, trust, purpose and access to help.
- Teams create norms for speaking up, support and recovery.
- Organisations remove systemic demands and learn from workforce evidence.

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TOPIC 02

Resilience ecosystem

1

Leadership

Visible priorities, psychologically safe behaviour and accountability.

2

Job design

Manage demand, control, staffing, role clarity and recovery time.

3

Social resources

Peer connection, manager support and trusted communities.

4

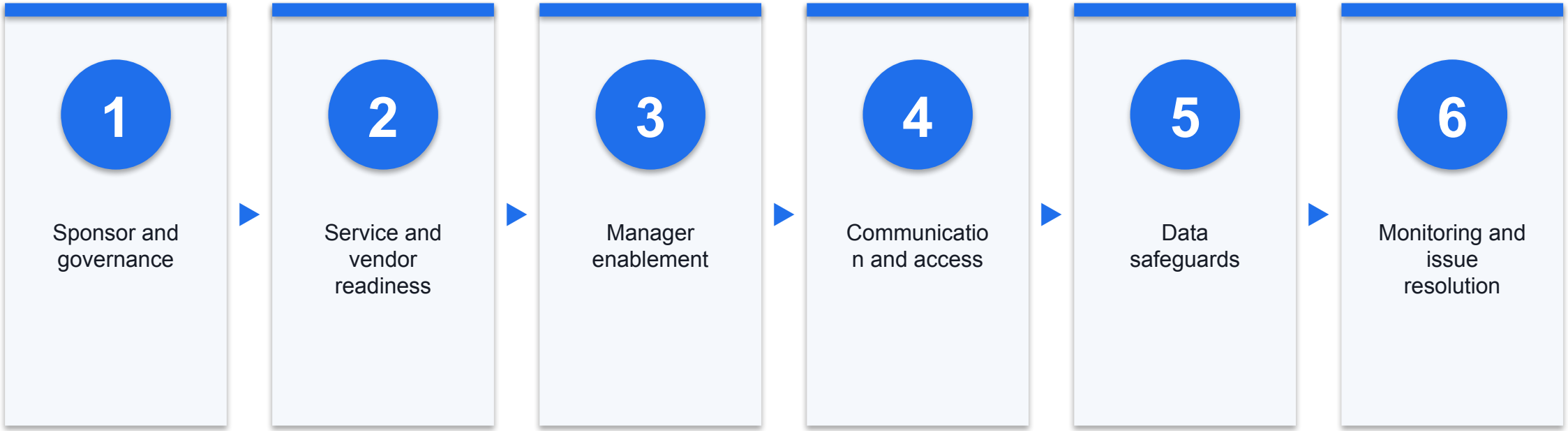
Professional support

Clear, confidential and timely access to qualified help.

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TOPIC 02

Implementation workstreams



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TOPIC 02

Roles and ownership

1

Sponsor

Sets priority, resources and accountability for outcomes.

2

Programme owner

Coordinates plan, risks, vendors, data and review.

3

Managers

Apply work practices, enable access and respond safely.

4

Employees and representatives

Co-design, participate voluntarily and provide feedback.

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TOPIC 02

Participation barriers

1

Awareness

Employees do not know what exists or why it is relevant.

2

Time and access

Shifts, workload, caregiving, location or technology block use.

3

Trust

Privacy, stigma or employment consequences feel uncertain.

4

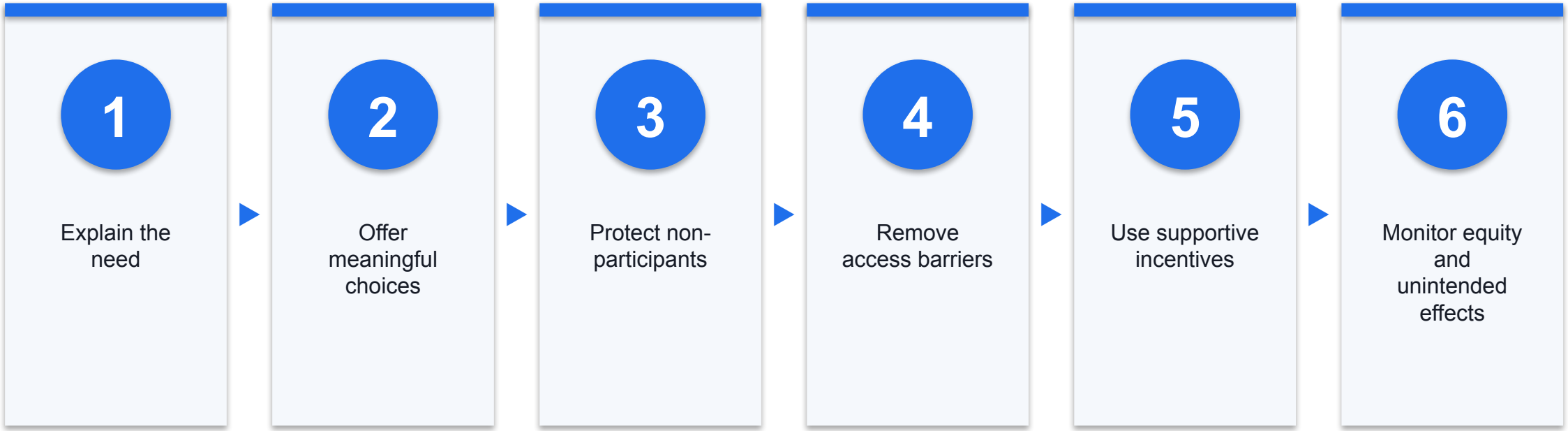
Perceived value

The offer does not match needs, culture or employee experience.

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TOPIC 02

Ethical engagement



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TOPIC 02

Partner and vendor governance

1

Capability

Qualified staff, service standards, escalation and clinical governance.

2

Privacy

Purpose, data flows, security, retention, transfer and breach duties.

3

Access

Hours, languages, channels, accessibility and capacity.

4

Evidence

Safe aggregates, service quality, outcomes and corrective action.

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TOPIC 02

Budget and value

Perspective A

- Budget for design, communication, manager time, access support and evaluation—not just vendor fees.
- Use a benefits logic and realistic time horizon.
- State uncertainty and avoid promising guaranteed ROI.

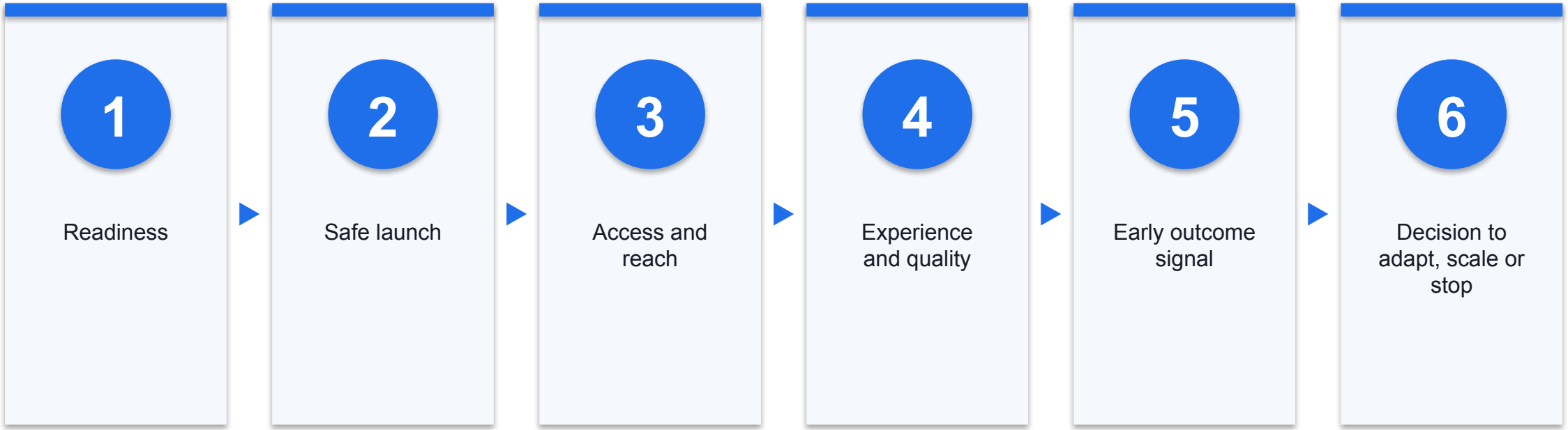
Perspective B

- Prioritise statutory and safety requirements first.
- Compare reach, equity, quality and outcome with cost.
- Scale only after a pilot demonstrates safe, relevant value.

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TOPIC 02

Pilot-to-scale gates



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TOPIC 02

Implementation is the programme.

Policy, vendor and launch event matter only when daily work, access and follow-through change.

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REAL-USE-CASE DECISIONS

Applied Activities — Programme Design and Implementation

1

Activity 4

Resilience Ecosystem After a Service Surge · LO2

2

Activity 5

Whole-Person Wellness Programme Portfolio · LO2

3

Activity 6

Mental-Wellbeing Policy-to-Practice Playbook · LO2

4

Activity 7

Participation and Partnership Recovery Plan · LO2

5

Evidence boundary

Published evidence and requirements are separated from fictional workplace inputs.

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Resilience Ecosystem After a Service Surge

ACTIVITY 4

Respond to fatigue and strain in a high-demand team without shifting responsibility onto employees to become tougher.

You'll build

Job Demands–Resources map, three-level intervention portfolio and 90-day implementation plan

Tools / evidence: CIPD evidence review · Workplace Mental Health framework · employee journey evidence

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ACTIVITY 4 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- CIPD's evidence review treats resilience as influenced by individual and contextual factors and cautions against placing the onus on employees to withstand harmful conditions. Protective factors include self-efficacy, positive affect, social support and leader relationships; effective development tends to be structured and experiential rather than a one-off lecture.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- CivicConnect and all employee evidence are fictional training inputs.
- There is no evidence that a one-hour webinar alone will address the primary work demands.
- The operating budget allows changes to rosters, escalation, peer support and a targeted development programme.

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ACTIVITY 4 · DISCUSS AND DECIDE

Activity Questions

1. Which demands are preventable, reducible or unavoidable?
2. Which resources are missing at individual, team and organisational levels?
3. What sequence balances immediate relief with sustainable redesign?
4. How will leaders know whether the change improves recovery and service?

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ACTIVITY 4 · ACCEPTANCE TEST

Resilience Ecosystem After a Service Surge

✅ Test it

The plan removes or reduces at least two preventable demands, adds team and organisational resources, treats individual training as complementary, includes safe escalation and has balanced employee/service measures.

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LEARNING UNIT 02 · LO2

Whole-Person Wellness Programme Portfolio

ACTIVITY 5

Convert a broad wellness ambition into a coherent prevention-to-recovery portfolio with access, ownership and evidence.

You'll build

Programme architecture, logic models and portfolio prioritisation map

Tools / evidence: VHA Whole Health evidence · HPB workplace resources · employee personas · portfolio canvas

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ACTIVITY 5 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- The published VHA Whole Health model emphasises whole-person goals and values, health coaching, wellbeing programmes and clinical care within a wider system. It illustrates how an ecosystem can orient support around what matters to the person rather than a fragmented menu of services.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- HarbourView, workforce data and current service performance are fictional.
- The organisation can use HPB resources where eligible but must validate current programme terms before commitment.
- Employee groups differ in schedule, physical demand, language, financial concerns and digital access.

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ACTIVITY 5 · DISCUSS AND DECIDE

Activity Questions

1. Which shared and targeted needs should the portfolio address?
2. How will prevention, early help, support and recovery connect?
3. What makes each pathway accessible and trustworthy for shift employees?
4. Which components should be piloted, scaled, partnered or stopped?

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ACTIVITY 5 · ACCEPTANCE TEST

Whole-Person Wellness Programme Portfolio

✅ Test it

Every component links to an evidenced need, the portfolio covers work conditions plus support, access is designed for distinct groups, clinical/privacy boundaries are explicit and each component has an owner and review measure.

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LEARNING UNIT 02 · LO2

Mental-Wellbeing Policy-to-Practice Playbook

ACTIVITY 6

Turn a generic mental-wellbeing policy into observable routines, decision rights, scripts and escalation paths.

You'll build

Manager playbook, process map and role-based implementation checklist

Tools / evidence: TAFEP guidance · manager scenarios · escalation map · implementation tracker

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ACTIVITY 6 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- Singapore's tripartite guidance encourages employers to strengthen mental wellbeing through organisation-, team- and individual-level practices, including leadership, supportive managers, work-life harmony, access to help and appropriate confidentiality.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- All characters and events are fictional learning scenarios.
- Managers are not clinicians and must not diagnose or provide therapy.
- The organisation has HR, an EAP and a documented emergency escalation route.

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ACTIVITY 6 · DISCUSS AND DECIDE

Activity Questions

1. Which policy statements need a concrete routine, tool or decision right?
2. What can managers ask, record, decide and escalate?
3. How should the playbook handle immediate safety concerns and ordinary work support differently?
4. What evidence demonstrates implementation beyond attendance at training?

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ACTIVITY 6 · ACCEPTANCE TEST

Mental-Wellbeing Policy-to-Practice Playbook

✅ Test it

The playbook covers routine through emergency situations, limits collection and diagnosis, names decision rights and handoffs, provides usable scripts and defines evidence of practice quality rather than training completion alone.

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Participation and Partnership Recovery Plan

ACTIVITY 7

Diagnose low engagement in an existing programme and redesign access, communication, incentives and vendor governance.

You'll build

Barrier map, vendor scorecard, budget reallocation and recovery experiment

Tools / evidence: NCBI programme principles · fictional utilisation funnel · vendor SLA · employee comments

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ACTIVITY 7 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- The NCBI review highlights multilevel leadership, organisational alignment, programme scope and quality, accessibility, partnerships and communications as core elements, while identifying awareness, time, manager support, access, privacy and perceived benefit as participation barriers.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- All funnel, budget, vendor and employee-comment data are fictional.
- Registration is not evidence of benefit and may be a vanity metric.
- The vendor can provide privacy-safe aggregates but the current contract lacks response-time and accessibility standards.

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ACTIVITY 7 · DISCUSS AND DECIDE

Activity Questions

1. Where does the participation funnel fail for each workforce group?
2. Which barriers are awareness, access, trust, relevance, manager or service-quality problems?
3. Would the proposed incentive support or distort the intended behaviour?
4. What partner standards and experiment would restore sustainable participation?

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ACTIVITY 7 · ACCEPTANCE TEST

Participation and Partnership Recovery Plan

✅ Test it

The plan diagnoses funnel loss by group, addresses at least three root-cause categories, avoids coercive incentives, sets vendor quality/privacy standards and defines a measurable experiment with owner and stop rule.

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LEARNING UNIT RECAP

Recap — Programme Design and Implementation

1. **Resilience Ecosystem After a Service Surge**
   Redesign job demands and resources while adding proportionate resilience support

2. **Whole-Person Wellness Programme Portfolio**
   Design an inclusive portfolio for distinct employee needs

3. **Mental-Wellbeing Policy-to-Practice Playbook**
   Translate policy into safe day-to-day manager and team practices

4. **Participation and Partnership Recovery Plan**
   Remove participation barriers and govern delivery partners for a sustainable programme

5. **Evidence habit**
   Separate public facts, organisation evidence and training assumptions.

6. **Management habit**
   Link every priority, risk or finding to an owner and review trigger.

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03

LEARNING UNIT 03

Communication, Feedback and Improvement

benefits · audience · channels · trust · feedback · metrics · management review

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KEY CONCEPTS

Key Concepts — Communication, Feedback and Improvement

1

Benefit

Describe a credible employee and organisational value proposition without overpromising.

2

Communication

Match message, messenger, channel, timing and call to action to each audience.

3

Feedback

Combine safe quantitative and qualitative evidence; make it easy to speak and see a response.

4

Improvement

Use a logic model, equity checks and management decisions to adapt or stop programmes.

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TOPIC 03

Trust is part of programme access.

People will not use support they believe could expose, label or disadvantage them.

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TOPIC 03

Benefits for employees and the organisation

Perspective A

- Easier access to relevant support and healthier work conditions.
- Greater capability, energy, connection and confidence to seek help.
- Fairer access across locations, shifts and life circumstances.

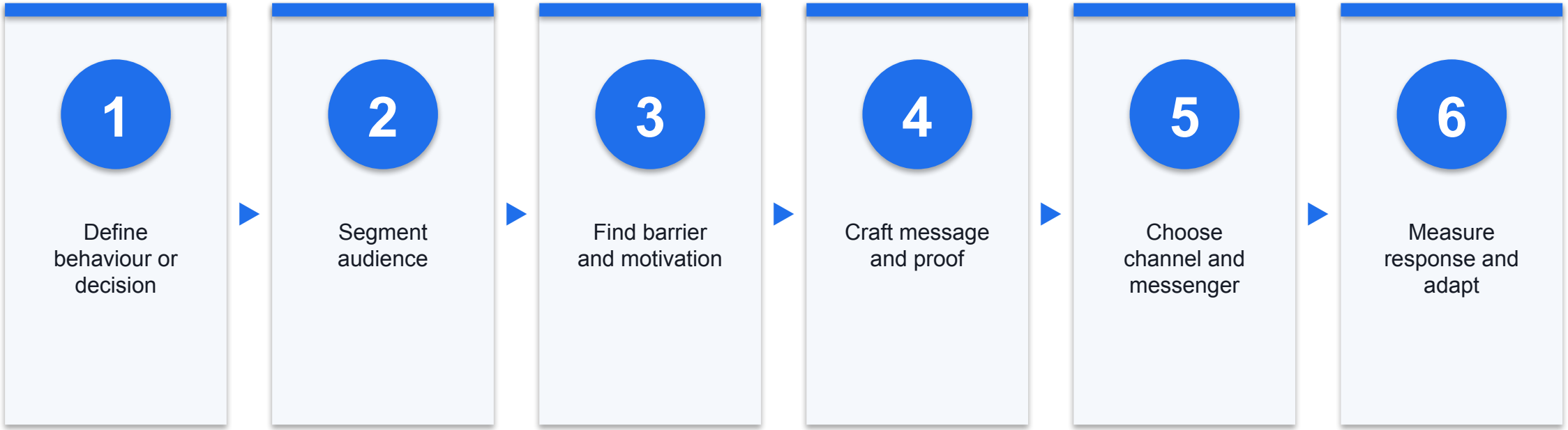
Perspective B

- Potential improvements in absence, retention, engagement and sustainable performance.
- Better risk visibility and manager capability.
- Benefits depend on context and implementation; association is not proof of causation.

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TOPIC 03

Communication design



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TOPIC 03

Audience differences

1

Desk-based and hybrid

Digital reach is high, but attention and isolation may be barriers.

2

Frontline and shift

Supervisor briefings, roster timing and paid access may matter more than email.

3

Managers

Need role clarity, scripts, boundaries and escalation—not only awareness.

4

Vulnerable or underserved groups

Require trusted messengers, accessible formats and stronger privacy assurance.

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TOPIC 03

Message architecture

1

Why

Name the employee need and intended benefit in plain language.

2

What

Explain the offer, boundaries and what it does not do.

3

How

Give a simple, accessible action and alternative path.

4

Trust

State privacy, voluntariness, provider and help or complaint route.

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TOPIC 03

Communication channels

1

Digital

Email, intranet, app, Teams, SMS and digital signage.

2

Human

Managers, champions, HR, union or employee representatives and peers.

3

Physical

Toolbox talks, posters, payslip inserts and onsite events.

4

Service moment

Onboarding, return-to-work, benefits enrolment and manager check-ins.

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TOPIC 03

Consistency and localisation

Perspective A

- Keep eligibility, privacy and access promises consistent.
- Use a single approved source for dates, contacts and service boundaries.
- Train messengers before launch.

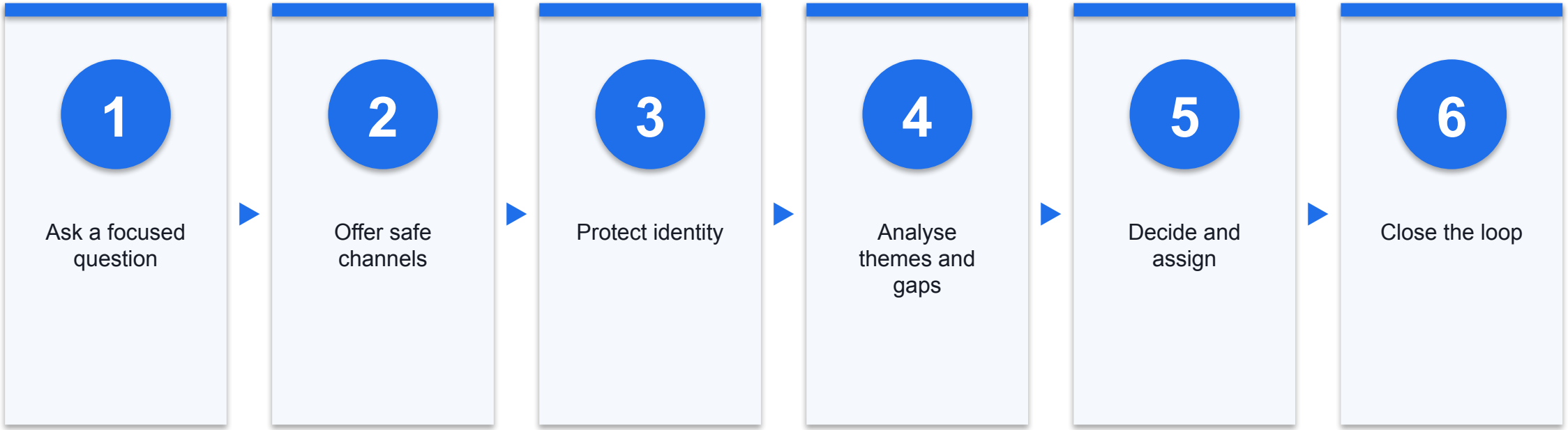
Perspective B

- Adapt examples, language, timing and channel to the audience.
- Use culturally relevant and accessible formats.
- Test comprehension, not merely message delivery.

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TOPIC 03

Employee feedback system



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TOPIC 03

Feedback methods

1

Survey

Scalable trends and segments; vulnerable to low response and design bias.

2

Focus group

Rich shared experience; power dynamics and confidentiality need facilitation.

3

Interview

Deep individual context; slower and harder to anonymise.

4

Programme and service data

Behavioural evidence; may not explain why people did or did not engage.

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TOPIC 03

Additional listening channels

1

Social listening

Use public or authorised data, clear purpose and ethical boundaries.

2

Exit and stay interviews

Reveal experience and retention factors; avoid treating departure as the only safe time to speak.

3

Suggestion and speak-up routes

Enable ongoing issues with acknowledgement and escalation.

4

Manager observations

Useful operational signal, but not a proxy for confidential employee voice.

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TOPIC 03

Anonymous and confidential

Perspective A

- Anonymous means identity is not collected or reasonably recoverable.
- Small teams, free text and metadata can defeat anonymity.
- Set minimum reporting cells and redact identifiers.

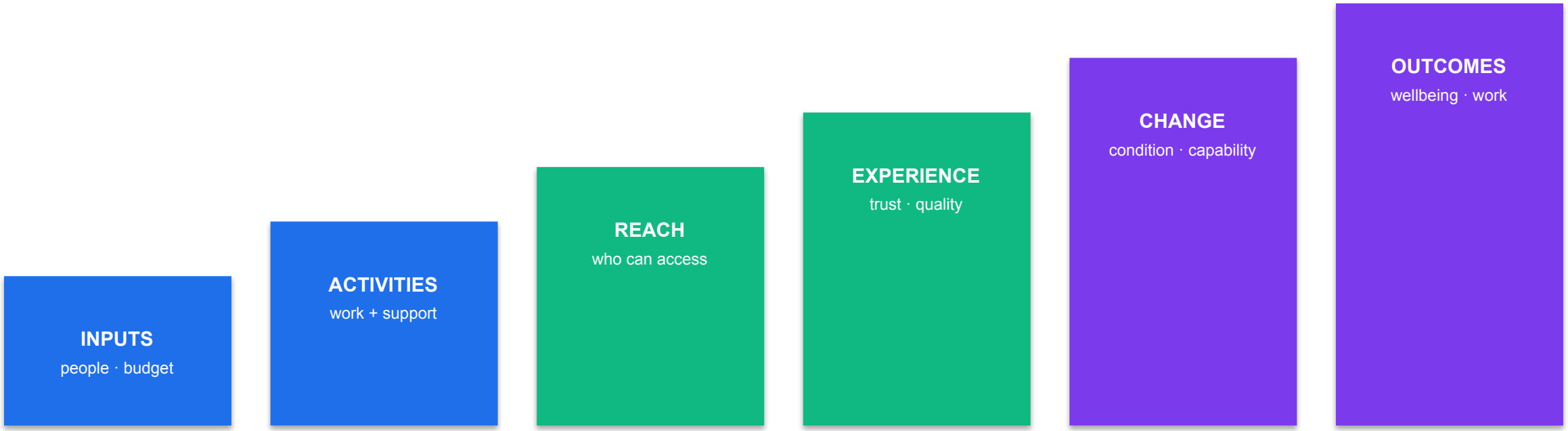
Perspective B

- Confidential means authorised people can know identity under defined limits.
- Explain who sees what and when escalation may occur.
- Do not promise anonymity when the channel is only confidential.

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EVIDENCE CHAIN

Programme logic model



Implementation evidence strengthens first → causal confidence requires design, comparison and time

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TOPIC 03

Balanced measures

1

Reach

Eligible employees, awareness and access by relevant group.

2

Participation

Uptake, repeat use, completion and drop-off.

3

Experience

Trust, relevance, accessibility, quality and psychological safety.

4

Outcome

Work conditions, wellbeing indicators and operational outcomes over time.

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TOPIC 03

Leading and lagging indicators

Perspective A

- Leading: manager capability, access, response time, workload controls and participation equity.
- They show conditions that can influence later outcomes.
- Use them for early correction.

Perspective B

- Lagging: absence, turnover, claims, incidents and sustained wellbeing outcomes.
- They are influenced by many factors and often change slowly.
- Avoid attributing change to one programme without suitable evidence.

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TOPIC 03

Evaluation threats

1

Selection bias

Participants may differ from non-participants before the programme.

2

Confounding

Workload, leadership, seasonality or policy changes influence outcomes.

3

Small numbers

Percentages may fluctuate and create privacy risk.

4

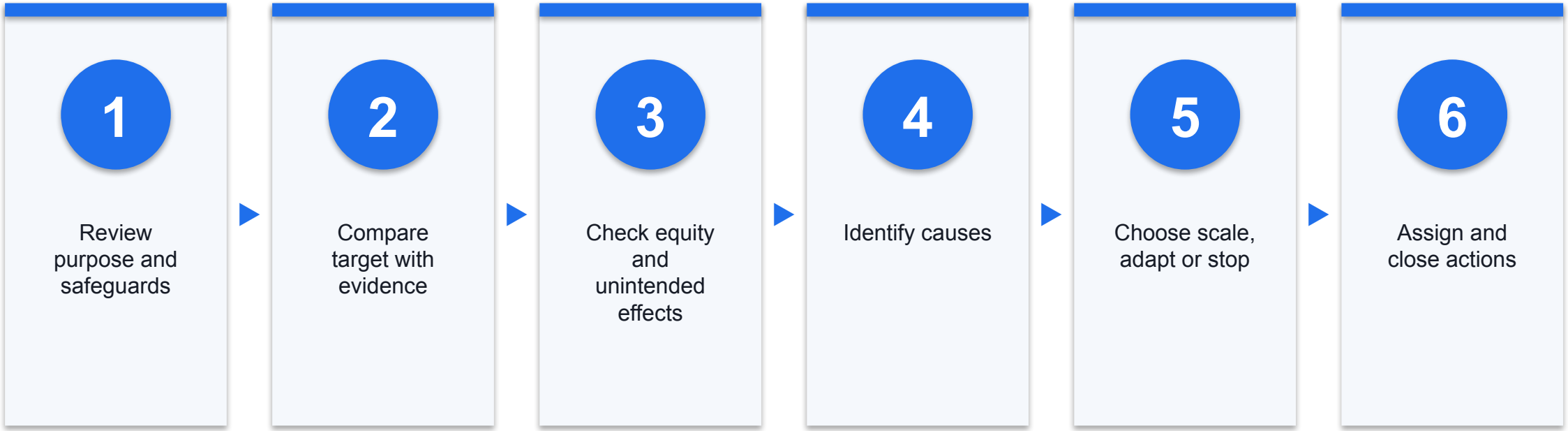
Vanity metrics

Registrations and clicks do not prove benefit or safe implementation.

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TOPIC 03

Management review



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TOPIC 03

Decision-ready report

1

Decision

What approval, resource, risk acceptance or change is required?

2

Evidence

What is known, how reliable is it and what remains uncertain?

3

Equity and risk

Who benefits, who is missing and what harm may occur?

4

Action

Owner, due date, measure, communication and next review trigger.

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TOPIC 03

Feedback earns trust only when people can see what changed.

Close the loop, explain constraints and keep measuring the employee experience.

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REAL-USE-CASE DECISIONS

Applied Activities — Communication, Feedback and Improvement

1

Activity 8

Inclusive Multi-Channel Wellbeing Campaign · LO3

2

Activity 9

Employee Feedback and Listening System · LO3

3

Activity 10

Wellbeing Programme Management Review · LO3

4

Evidence boundary

Published evidence and requirements are separated from fictional workplace inputs.

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Inclusive Multi-Channel Wellbeing Campaign

ACTIVITY 8

Design a trust-centred campaign that reaches office, frontline and mobile employees without overclaiming outcomes.

You'll build

Audience-message-channel matrix, campaign assets plan and measurement framework

Tools / evidence: Gallup wellbeing framing · employee personas · channel data · message testing sheet

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ACTIVITY 8 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- Gallup's research treats career wellbeing as foundational and describes employee wellbeing as connected with engagement, absence, performance and retention. These relationships can guide a value proposition but do not prove that a single campaign or programme causes each outcome.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- Personas, channel reach, trust scores and message-test results are fictional.
- EAP individual use is confidential to the provider, subject to stated safety and legal boundaries.
- The campaign has six weeks and a S$18,000 budget.

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ACTIVITY 8 · DISCUSS AND DECIDE

Activity Questions

1. What behaviour or decision should each audience take?
2. Which benefit, barrier, messenger and channel combination fits each group?
3. How will the campaign explain privacy and voluntariness accurately?
4. Which measures distinguish delivery, comprehension, trust and action?

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ACTIVITY 8 · ACCEPTANCE TEST

Inclusive Multi-Channel Wellbeing Campaign

✅ Test it

Every priority audience has a defined action, credible benefit, trusted messenger and accessible channel; privacy is accurate; overclaiming is absent; and measures cover reach, comprehension, trust, action and equity.

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Employee Feedback and Listening System

ACTIVITY 9

Design a feedback system that combines surveys, focus groups, interviews and programme data while protecting identity and closing the loop.

You'll build

Feedback architecture, survey/facilitation instruments and response protocol

Tools / evidence: Workplace Mental Health resilience framework · sample dataset · channel risk cards · reporting template

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ACTIVITY 9 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- The Center for Workplace Mental Health describes organisational resilience through leadership, training, a culture of empowerment, purpose, trust and accountability, supportive work environments, flexibility and access to support. Employee voice is needed to understand whether those conditions are experienced in practice.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- All sample responses and organisational details are fictional.
- Reporting groups must meet a minimum threshold and free text needs disclosure review.
- An independent research administrator is available for confidential interviews.

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ACTIVITY 9 · DISCUSS AND DECIDE

Activity Questions

1. Which question requires survey, focus group, interview or service data?
2. What data should not be collected, and what reporting threshold is safe?
3. How will employees understand anonymity, confidentiality and escalation?
4. How will management acknowledge, decide and close the feedback loop?

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ACTIVITY 9 · ACCEPTANCE TEST

Employee Feedback and Listening System

✅ Test it

The system collects only decision-relevant data, uses complementary methods, protects small groups and free text, accurately distinguishes anonymity/confidentiality, includes safety escalation and commits to visible response timelines.

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LEARNING UNIT 03 · LO3

Wellbeing Programme Management Review

ACTIVITY 10

Turn reach, experience, work-condition, outcome and cost evidence into a decision-ready management review.

You'll build

Programme dashboard, evidence confidence assessment and management decision paper

Tools / evidence: McKinsey thriving-at-work research · fictional 12-month dataset · logic model · decision template

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ACTIVITY 10 · REAL EVENT

Case Scenario — Public Facts and Training Guardrails

Reported / sourced

- McKinsey's workplace research presents wellbeing as a leadership priority and highlights organisational enablers such as psychological safety, purpose and adaptable support. The source supports a systems perspective; the fictional programme dataset must be evaluated on its own quality and cannot inherit causal conclusions.
- Full source is recorded in speaker notes and the Learner Guide.

Training assumptions / limits

- All programme, outcome, cost, baseline and comparison data are fictional.
- Business units were not randomly assigned; workload and leadership changes occurred during the pilot.
- Small groups and sensitive service data must be aggregated before management reporting.

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ACTIVITY 10 · DISCUSS AND DECIDE

Activity Questions

1. Did delivery reach the intended groups safely and equitably?
2. What evidence supports changes in experience, work conditions and outcomes?
3. Which alternative explanations and data-quality weaknesses limit confidence?
4. What should management scale, adapt, pause or stop, and what is the next review trigger?

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ACTIVITY 10 · ACCEPTANCE TEST

Wellbeing Programme Management Review

✅ Test it

The review validates data, follows the logic model, separates implementation from outcome, analyses equity, names confounders, avoids causal overclaiming and gives component-level decisions with owners and review triggers.

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LEARNING UNIT RECAP

Recap — Communication, Feedback and Improvement

1

Inclusive Multi-Channel Wellbeing Campaign

Communicate credible programme benefits through appropriate techniques and channels

2

Employee Feedback and Listening System

Gather safe, representative employee feedback using complementary methods

3

Wellbeing Programme Management Review

Assess programme effectiveness and recommend scale, adaptation or stop

4

Evidence habit

Separate public facts, organisation evidence and training assumptions.

5

Management habit

Link every priority, risk or finding to an owner and review trigger.

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WRAP-UP

Wellbeing Is Designed, Enabled and Improved

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COURSE SUMMARY

What You Achieved

1

LO1

Identify workplace health and wellness trends, technologies and research implications for employees and the organisation.

2

LO2

Implement inclusive health and wellness programmes for different employee groups and translate policies into day-to-day practices.

3

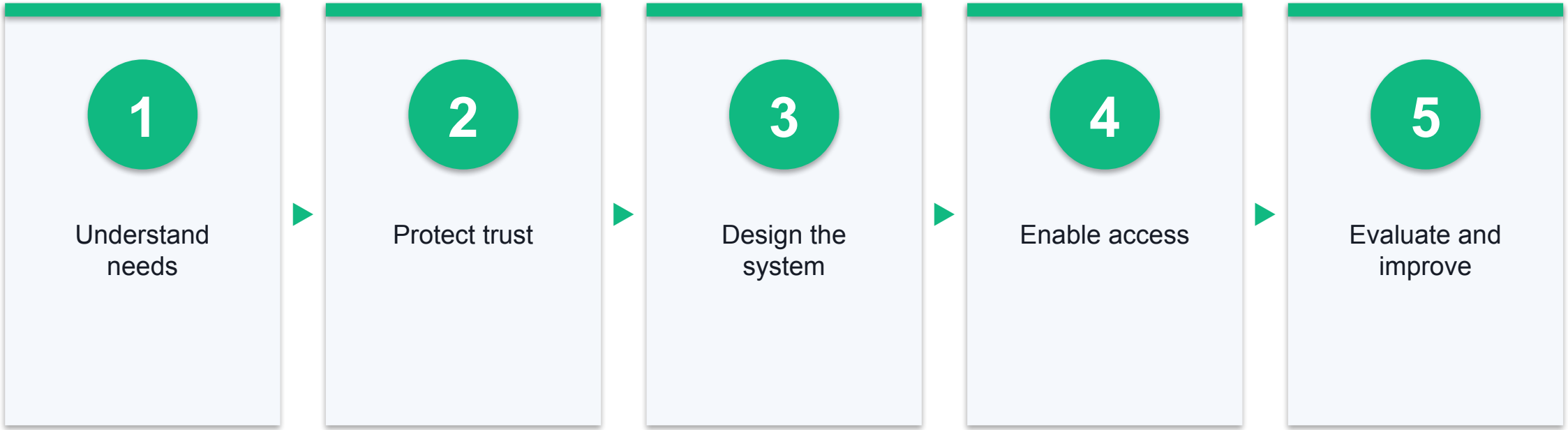
LO3

Communicate programme benefits through appropriate channels and assess effectiveness using employee feedback and programme evidence.

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TAKEAWAY

The complete programme discipline



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SOURCE MAP · FULL URLS IN NOTES AND LEARNER GUIDE

Research Foundations

1

NCBI

Programme design, accessibility, partnerships and participation barriers.

2

Gallup

Whole-person wellbeing elements and employee-experience relationships.

3

CIPD

Resilience evidence, protective factors and system responsibility.

4

Workplace Mental Health

Leadership, culture, flexibility and access to support.

5

McKinsey

Thriving-at-work research and organisational enablers.

6

Peer-reviewed studies

Resilience ecosystems and whole-person care implementation.

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SOURCE MAP · VALIDATED 16 AUGUST 2026

Singapore Requirements and Support

1

PDPC

Personal-data obligations and privacy-by-design baseline.

2

TAFEP

Mental-wellbeing and fair, supportive employment practices.

3

HPB

Current workplace-health resources and programme guidance.

4

MOM

Employment Act sick-leave eligibility and administration.

5

Healthier SG

National preventive-health and community-support pathway.

6

Course page

Course outcomes, topics and duration.

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SOURCE MAP · APPRAISE BEFORE APPLYING

Practice Perspectives

1

W Talent Solutions

Whole-person wellness and workplace resilience examples.

2

Outback Team Building

Activity examples treated as practice inspiration, not causal evidence.

3

Evidence hierarchy

Regulation and research carry more weight than vendor or promotional claims.

4

Decision rule

Use source quality, relevance, workforce evidence and safe piloting together.

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Course Feedback

1

Scan

Scan the course feedback QR code shown by the trainer.

2

Rate

Rate the content, activities and trainer delivery.

3

Suggest

Tell us what would make the course more useful at work.

4

Certificate

Your certificate of completion is emailed after the course.

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WORKPLACE WELLBEING

Thank You

Design healthy work, earn trust and close the learning loop with evidence.

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